

15.0 OSHA INSPECTION

Purpose

To establish the policy for all managers to follow if an OSHA Compliance inspection will be conducted.

Overview:

The Occupational Safety and Health Administration (OSHA) is authorized to conduct workplace inspections to determine whether employees are complying with standards issued by the agency for safe and healthful workplaces. Many States have their own occupational safety and health programs, and regularly inspect workplaces. Inspections are usually conducted without advance notice and can be conducted for one or more of the following reasons:

Imminent Danger Situations – Any condition where there is reasonable danger that a situation exists that can be expected to immediately cause death or serious harm. **Catastrophes and Fatal Accidents** – Investigation of fatalities and accidents resulting in the hospitalization of 3 or more employees. Such catastrophes must be reported to OSHA within 8 hours.

Employee Complaints

Programmed Inspections – Based on injury rates, previous citation history, and employee exposure to toxic substances or random computerized selection.

This policy details the phases of an OSHA compliance inspection, the response and attitude of management to an inspection and steps to insure completion of the appropriate follow-up corrective action.

Policy:

EAFab Corporation policy is to demonstrate “good faith” effort to comply with all OSHA standards and any health and safety issues raised in an OSHA compliance inspection.



SAFETY AND HEALTH PROGRAM

Management is responsible for implementing this policy and correcting all health and safety deficiencies revealed during compliance inspections. Pedro Quiroga will provide technical assistance and coordination of corrective action, as required.

Admitting an OSHA Compliance Officer

If an OSHA compliance inspector requests to conduct an inspection, the senior management member is to ask to see the officer's credentials.

An OSHA inspector carries either U.S. or the state's Department of Labor credentials bearing their photograph and a serial number. In every case, verify the authenticity of the compliance inspector's identification by calling the nearest OSHA office.

Note: DO NOT REFUSE THE COMPLIANCE OFFICER ADMITTANCE.

The senior management member is to contact Pedro Quiroga immediately.

If EAFab Corporation requires a Search Warrant, inform the OSHA compliance officer before the opening conference begins. EAFab Corporation rights to challenge a warrant may be lost if it permits the inspection to proceed.

OSHA Facts

An OSHA Inspection is divided into three parts:

The Opening Conference

The Walk Around Inspection

The Closing Conference

There are no time limits specifying how long an inspector may remain on the premises. Violations are considered to be "alleged violations" until they become a final order of the Occupational Safety and Health Review Commission.

EAFab Corporation may contest (appeal), in writing any part of the citation within 15 working days after it has received it.

The citation must be posted in the work place for three days following its receipt or until the condition creating the alleged violation is corrected. Management will ask for clarification about any point(s) an inspector raises that they don't understand. Management and employees will not admit to violating any safety standard.

If EAFab Corporation contests (appeals) an alleged violation, copies of the appeal will be posted at the work site.

Opening Conference

Before inspecting the premises, the OSHA compliance officer will conduct an opening conference at which they will explain:

The reason for the inspection (for example. employee or individual complaint) Purpose of the visit

Scope of the inspection

OSHA Standards that apply

The below are listings of all OSHA Standards

OSHA Standards

1904, Recording and Reporting Occupational Injuries and Illnesses

1904 Table of Contents/Authority for 1904

- 1904.1, Purpose and scope.
- 1904.2, Log and summary of occupational injuries and illnesses.
- 1904.3, Period covered.
- 1904.4, Supplementary record.
- 1904.5, Annual summary.
- 1904.6, Retention of records.
- 1904.7, Access to records.
- 1904.8, Reporting of fatality or multiple hospitalization incidents.
- 1904.9, Falsification, or failure to keep records or reports.
- 1904.10, Recordkeeping under approved State plans.
- 1904.11, Change of ownership.
- 1904.12, Definitions.
- 1904.13, Petitions for record keeping exceptions.
- 1904.14, Employees not in fixed establishments.
- 1904.15, Small employers.
- 1904.16, Establishments classified in Standard Industrial Classification Codes (SIC) 52-89,
(except 52-54, 70, 75, 76, 79 and 80).
- 1904.17, Annual OSHA Injury and Illness Survey of Ten or More Employers.
- 1904.20, Description of statistical program.
- 1904.21, Duties of employers.
- 1904.22, Effect of State plans.
- 1904.30, OMB control numbers under the Paperwork Reduction Act.
- Other OSHA Standards with Recordkeeping Requirements
- 1910.95, Occupational noise exposure
- 1910.120, Hazardous waste operations and emergency response
- 1910.440, Recordkeeping requirements
- 1910.1000, Toxic & Hazardous Substances
- 1910.1001, Asbestos

1910.1018, Inorganic arsenic

1910.1025, Lead

1910.1027, Cadmium

1910.1028, Benzene

1910.1029, Coke oven emissions

1910.1030, Bloodborne pathogens

1910.1043, Cotton dust

1910.1044, 1,2-dibromo-3-chloropropane

1910.1045, Acrylonitrile

1910.1047, Ethylene oxide

1910.1048, Formaldehyde

1910.1050, Methylenedianiline

1910.1051, 1,3-Butadiene

1910.1052, Methylene Chloride

1910.1450, Occupational exposure to hazardous chemicals in laboratories

1913.10, Rules of agency practice and procedure concerning OSHA access to employee medical records

1915.7, Competent person

1915.1001, Asbestos

1919.11, Recordkeeping and related procedures concerning records in custody of accredited persons

1919.12, Recordkeeping and related procedures concerning records in custody of the vessel.

1925.3, Records

1926.60, Methylenedianiline

1926.62, Lead

1926.65, Hazardous waste operations and emergency response

1926.800, Underground Construction

1926.1091, Recordkeeping requirements

1926.1101, Asbestos

1926.1127, Cadmium

1960, Federal employees

1960.66, Purpose, scope and general provisions

1960.67, Log of occupational injuries and illnesses

1960.68, Supplementary record of occupational injuries and illnesses

1960.70, Reporting of serious accidents

1960.71, Locations and utilization of records and reports

1960.72, Access to records by Secretary

1960.73, Retention of records

1960.74, Agency annual reports

Preambles to OSHA Standards

Reporting of Fatality or Multiple Hospitalization Incidents.

OSHA Directives

CPL 2.80, Handling of Cases To Be Proposed for Violation-By-Violation Penalties, (1990, October 21), 15 pages. Includes procedures for record keeping violations.

CPL 2.91, Enhanced Verification of Records, (1990, May 13), 6 pages.

CPL 2-2.46, 29 CFR 1913.10(b)(6), Authorization and Procedures for Reviewing Medical Records, (1989, January 5), 5 pages.

CPL 2-2.33, 29 CFR 1913.10, Rules of Agency Practice and Procedure Concerning OSHA Access to Employee Medical Records - Procedures Governing Enforcement Activities, (1982, February 8), 12 pages.

CPL 2-2.32, 29 CFR 1913.10(b)(6), Authorization of Review of Specific Medical Information, (1981, January 19), 5 pages.

CPL 2-2.30, 29 CFR 1913.10(b)(6), Authorization of Review of Medical Opinions, (1980, November 14), 2 pages.

CPL 2.113, Fatality Inspection Procedures, (1996, April 1), 5 pages.

Review Commission Decisions

81-2135, (1985, April 17), 5 pages. Failure to make records available during an inspection.

82-630, (1991, February 15), 9 pages. Making medical records available when a Workers Compensation claim is pending.

82-1016, (1987, March 18), 7 pages. Privacy of OSHA 200 and related records.

89-2614, (1993, February 3), 8 pages. Recording of elevated blood lead levels on the OSHA 200.

SAFETY AND HEALTH PROGRAM

90-552, (1992, February 21), 2 pages. OSHA 200 must be maintained at each location.

89-433, (1993, April 27), 9 pages.

90-2179, (1993, April 1), 3 pages. Assessing separate penalties for multiple errors on the

OSHA 200

87-0922, (1993, February 5), 25 pages.

88-237, (1994, May 23), 6 pages.

91-0110, (1996, January 19), 6 pages.

Standard Interpretations and Compliance Letters

There are several hundred Standard Interpretations and Compliance Letters relating directly to the topic of Recordkeeping.

Please refer to the Search Page on the "<http://www.OSHA.gov>" web site. From here you can access these documents either by the specific regulation, or by conducting a search.

You can also contact OSHA at one of their regional offices. They will provide you with forms and answers to any questions you may have. Don't hesitate to use them as a valuable resource.

Senior Management must arrange for the following to attend the opening conference: Pedro Quiroga of EAFab Corporation, Other Personnel, as directed

Management must request copies of all applicable safety and health standards as well as a copy of any employee complaint.

The Walk Around Inspection

After the opening conference, the OSHA compliance officer will go through the facility to inspect for safety and health hazards. At a minimum, the OSHA compliance officer will likely ask for documentation of the following:

Compliance with the hazard communication standard. Compliance with the lockout/ tagout standard.

Record keeping for employee training

The employee written safety and health management program

When senior management members and other EAFab Corporation employees accompany an OSHA compliance officer on an inspection, they should be respectful while firmly standing up for EAFab Corporation rights and viewpoints. The conduct of EAFab Corporation personnel shall be in accordance with the following guidelines:

Do not physically interfere with the OSHA compliance officer when they are making the inspection

Do not give false or misleading information.

Accompany the OSHA compliance officer at all times during the inspection.

Answers to an OSHA compliance officer's questions are to be responsive to the question asked. Do not offer any information beyond the scope of the question.

Avoid making any statement that could be construed as an admission of a violation of any recognized health standard.



SAFETY AND HEALTH PROGRAM

Do not discuss with the OSHA compliance officer any previous safety inspections.

If the OSHA compliance officer wants to take photographs, senior management must request copies of the photographs. Senior management will also take photographs of the area from the same and different angle.

Watch and take notes regarding all activities of the OSHA compliance officer. Notes should be detailed and should include such pertinent information as to the name(s) of the OSHA compliance officer(s), time of arrival, activities of OSHA compliance officer, amount of time spent at each location, comments about violations and potential citations, who was interviewed, what was said, etc.

Immediately correct minor but apparent safety problems in order to help establish EAFab

Corporation "good faith" effort to comply with all OSHA health and safety standards.

The OSHA compliance officer cannot and will not act in a consultative capacity. If they see or if EAFab Corporation personnel points out a violation, the OSHA compliance officer must issue a citation.

Closing Conference

After the walk around inspection, a closing conference is held with the OSHA compliance officer, senior management, and any employee representative. The OSHA compliance officer will discuss all unsafe and unhealthy situations observed and will identify all applicable sections of the standards which may have been violated.

Management will insure that all violations are understood. When appropriate, Management will produce records to show compliance efforts and fully explain any difficulties that will be encountered in the correction of safety hazards. Management and employees will not admit violation or indicate how long it will take to correct a potential violation.

Post Inspection Activities

Time limits to correct violations generally range from 5 to 30 days, unless an extension is requested. Time limits will be given in person at the closing conference or mailed within 30 days in a written report of the inspection findings.

Follow-up action will be documented in writing, by senior management, listing specific action steps, the individual accountable, and the target date for completion. Management is responsible for completing all corrective action.

OSHA inspection reports, EAFab Corporation response, and all correspondence to and from

OSHA will be retained permanently by Pedro Quiroga.

Questions an OSHA Compliance Officer Might Ask in an Administrative Interview:

1. Do you have a written Hazard Communication Plan?

29CFR 1910.1200 requires employers to have a written plan which describes how the training, labeling, SDS management and other requirements of "Right-to-Know" will be met. More citations and fines are given for this than anything else.

2. Do you have a complete written inventory (list) of hazardous materials?

29CFR 1910.1200 requires employers maintain a current list of all hazardous materials used in the workplace. This list must be accessible to employees.

3. Has a specific person been assigned responsibility for your safety program?



SAFETY AND HEALTH PROGRAM

29CFR 1910.1200 and other regulations require that you assign responsibility for various aspects of the safety program. Some states specifically require that employers name a person with overall safety responsibility.

4. Do you have a formal disciplinary policy relating to safety?

29CFR 1910. Various sections require employers enforce safety rules. Employees may not decide on their own when to follow the rules.

5. Do employees ever complain of headaches, nausea, dizziness or skin problems?

All OSHA standards require that employers evaluate workplace hazards and determine whether material use or employee complaints mean that there is any over-exposure to unsafe conditions.

These are typical symptoms of over-exposure.

6a. Do employees wear respirators or dust masks?

6b. If "Yes": Do you have written respirator procedures?

29CFR 1910.134 requires that if any employee uses a respirator, including a dust mask, written procedures must cover use, fit testing, cleaning and maintenance of the respirator.

6c. Do you have records showing fit testing of respirators and training?

29CFR 1910.134 requires employers to test the fit of each respirator on each employee and train the employee to check and properly use the respirator.

7. Do you have written training records?

29CFR 1910.1200, .1450, .1030 and virtually all other OSHA regulations require written training records which document date, subject, attendees and trainer.

8a. Do you have more than 10 employees?

8b. If "Yes": Do you have a written Emergency Contingency Plan?

29CFR 1910.38 outlines the requirements for an emergency contingency plan for those who employ more than 10 at any one time during the year.

8c. Are your Form OSHA Logs up-to-date and posted Feb 1 until April 30?

29CFR 1904 requires that employers of more than 10 at any one time in the year maintain occupational illness and injury reports on Form 301 or equivalent and summarize them on Form OSHA Log which is posted each Feb. 1 until April 30.

9a. Can you reasonably anticipate that any employees will be exposed to human blood this year because of their jobs?

9b. Have you assigned responsibility for first-aid to an employee?

9c. If "Yes": Do you have written Bloodborne Pathogen Exposure Control Plan?

9d. Have employees been trained in protective equipment and procedures?

29CFR 1910.1030 requires that employers develop an Exposure Control Plan, train employees, keep records, and offer Hepatitis B vaccinations if it can be reasonably anticipated that one or more employees could be exposed to human blood or

SAFETY AND HEALTH PROGRAM

blood products as a result of doing their assigned duties. If you have assigned first aid responsibilities to an employee you are required to have a Bloodborne Pathogen Program. Special waste management and use of approved disinfectants are also required. The key is "reasonable anticipation". Good Samaritan acts are not covered.

Janitorial & Chemical Storage Area Overview

10. Is the area neat and clean, without spills on the floor?

29CFR 1910.22 requires that all work places be clean, orderly and sanitary.

11. Are there any containers without legible labels?

12. Do all secondary container labels list the product, the hazards and the manufacturer?

29CFR 1910.1200 requires that all containers of hazardous materials be labeled. The manufacturer's label is fine if legible. If materials are moved from the original to a "secondary" container, it must be labeled. The label must include the name of the material, a description of the hazard and the manufacturer's name. Just the name is not enough.

13. Is there an SDS on hand for each hazardous material?

14. Are SDSs accessible to all employees at all times?

15. Pick a product. Ask to see the SDS. Could an employee have found it in 4-5 minutes?

29CFR 1910.1200 requires that employers have an SDS for each hazardous material. Employees must have access to SDS's at all times during the work shift and be able to find a specific one in less than 5 minutes without asking for access to the collection.

General Work Areas Overview

16. Is the fire extinguisher tag marked for monthly inspections and service in the last year?

29CFR 1910.157 requires that all portable fire extinguishers be visually inspected monthly and serviced annually. If the tag isn't marked it is difficult to prove inspections.

17. Is the area clean and uncluttered?

29CFR 1910.22 requires that all work places be clean, orderly and sanitary.

18. Are oily rags kept anywhere but in metal cans with closed lids?

29CFR 1910.38 requires employers to identify and correct fire hazards. Oily rags should be kept in a closed metal container.

19. Are coffee, drinks or food kept near any hazardous materials?

29CFR 1910.142 requires that no employee be allowed to have food or beverages in an area where they could be contaminated with toxic or infectious materials.

20. Are there any unlabeled containers?

29CFR 1910.1200 requires that all containers of hazardous materials be labeled. The manufacturer's label is fine if legible. If materials are moved from the original to a "secondary" container, it must be labeled. The label must include the name of the material, a description of the hazard and the manufacturer's name. Just the name is not enough.

21. Are any respirators stored which are not in bags or cabinets?

SAFETY AND HEALTH PROGRAM

29CFR 1910.134 requires that respirators be stored and maintained in a way that they will be cleaned, protected and ready for use. Respirators left in the open may absorb contaminants and become unusable.

22. Are gloves, goggles or safety glasses clean and in good repair?

29CFR 1910.132 requires that safety equipment be maintained in clean and sanitary condition and that it be used only if in good repair. Broken or dirty equipment raises questions in an inspector's mind and leads to a more intensive inspection.

23. Are there extension cords across aisles or walkways?

29CFR 1910.22 requires that all work place be clean, orderly and sanitary. Cords across aisles present a slip and fall hazard as well as a potential electrical hazard.

24. Look at ladders. Are there broken steps or parts in bad repair?

29 CFR 1910.25 requires employers to "inspect ladders frequently and those which have developed defects shall be withdrawn from service for repair or destruction and tagged or marked as "Dangerous, Do Not Use"."

25. Are there any broken or missing electrical switch or outlet covers?

29 CFR 1910.305 requires that pull boxes, junction boxes and fittings have plates or covers. Broken plates and covers do not provide adequate protection.

Employee Area Overview

26. Is the OSHA Poster or state equivalent posted?

27. Are emergency phone numbers posted by telephones?

28. Is an evacuation route map posted?

29 CFR 1910.38.

29. Is there a fully stocked first aid kit?

29 CFR 1910.262 requires that there be a first aid kit stocked with supplies appropriate to the situation. It must be continuously stocked for any emergency.

30. Are lunches, snacks or drinks stored in a cabinet or refrigerator with chemicals?

29 CFR 1910.142 requires that no employee be allowed to have food or beverages in an area where it could be contaminated with toxic or infectious materials.

Employee Interview

OSHA uses "performance based" standards for its enforcement of safety regulations. The best program on paper will mean nothing if your employees cannot do the right thing or do not know where to get information. Whether your employees can answer questions correctly (or not) is the test OSHA inspectors use to evaluate your compliance with OSHA rules.

31a. Please show me the SDS for (name a product) . Did the employee answer -- "What's an SDS?"

31b. Did the employee know where the SDS's are kept?

31c. Did it take less than 5 minutes for the employee to find the correct SDS?

SAFETY AND HEALTH PROGRAM

29 CFR 1910.1200 Employees should know what an SDS is and be able to locate a specific one in less than 5 minutes. SDS's should be indexed and stored in an organized fashion.

32a. When were you last trained on safety issues? Did the employee say "I don't remember" or "Never"?

32b. Has training been in the last year?

29 CFR 1910.1200 states that "employers shall provide information and training on hazardous chemicals...at the time of their initial assignment and whenever a new hazard is introduced into their work area." Some states also specifically require annual retraining.

33. If you had to evacuate the building where would you go for a head count? Did the employee know a pre-determined specific place?

29 CFR 1910.38 requires that emergency contingency plans specify the means of accounting for all employees after an evacuation of the facility.

OSHA Recordkeeping and Posting Requirements

Purpose

To establish the policy and procedures regarding EAFab Corporation requirements for compliance with OSHA record keeping and posting guidelines for occupational injuries and illnesses.

Policy

All locations are to post the "Job Safety and Health Protection" poster (or state equivalent) in prominent places in the workplace.

OSHA requires that employers maintain a record of certain occupational injuries that occur at each business establishment on the OSHA Form Log 300 and 300A: Log of Work-Related Injuries and Illnesses and Summary of Work-Related Injuries and Illnesses. At the end of each year, OSHA requires the summary section of the OSHA Form Log 300A to be posted at each business establishment no later than February 1 and remain in place until April 30. EAFab Corporation will comply with this requirement. Pedro Quiroga is responsible for maintaining the information on the log in a current status and distributing the OSHA Form Logs.

The "Job Safety and Health Protection" poster and the Form Log and Summary of Occupational

Injuries and Illnesses can be ordered from OSHA, free of charge, at 303-844-1600

Record Retention

OSHA Form Log, January – November reports can be discarded upon receipt of the next monthly report.

Year-end OSHA Form Log 200, 300, 300A, and 301, retain for 5 years following the year to which they relate.

Common OSHA Violations:

- Failing to provide information about the Hazard Communication standard and the actual hazards of the chemical that are present.
- Not having a Hazard Communication Program. Not having a written fire prevention program.
- OSHA Log hasn't been properly maintained or is missing.
- Not having an SDS for every hazardous chemical in use.
- Not properly labeling all containers or groups of containers containing hazardous chemicals.
- Not marking exits or accesses to exits.

SAFETY AND HEALTH PROGRAM

- Improper building design, construction, maintenance or occupancy of a building or structure containing employees.
- Fire extinguishers not located or mounted in an accessible and safe location or not provided.
- Failure to provide fire extinguisher training.
- Improper wiring is present in one of the following ways: Unused openings and electrical boxes not closed. Conductors entering boxes are not protected from abrasion Improperly using a flexible cord in one of the following ways: Flexible cord smaller than a #12 was spliced
 - Solder used to splice a flexible cord
 - Used as a substitute for fixed wiring
 - Ran through holes in the ceiling and/or walls
 - Ran through doorways and/or windows
- Exposed or non-current carrying metal surfaces of fixed equipment are not grounded. Failing to provided electrical boxes and fittings with an approved cover, or failing to ground metal covers.
- Disconnects, circuit breakers, and other over-current devices aren't legibly and permanently labeled.

Common OSHA Violations (cont.):

- Tongue guard on grinder is more than ¼" from the edge of the stone. Missing or inadequate machine guarding.
- Work rest is missing or more than 1/8" from a grinding wheel. Not providing a suitable eyewash or shower.
- Persons without respirators performing tasks that require respirators.
- Written standard operating procedures governing the use and selection of respirators shall be established.
- Employers shall make conveniently available protectors suitable for the task to be performed. Protective eye, head, face, body, feet and hand equipment shall be provided when there is reasonable probability of injury.
- A Platform four feet or more from the ground is not provided with a standard railing (and toe board) where required.
- Broken or damaged ladders being used.
- Furniture, barrels, boxes, or other devices used in lieu of ladders.

BLOODBORNE PATHOGENS

It is imperative that management makes available the following information to all employees during a training session. All employees shall be trained on the risk of bloodborne pathogens and the proper handling of blood and other bodily fluids.

What everyone needs to know:

- Bloodborne pathogens are microorganisms carried by human blood (and other body fluids) and cannot be seen with the naked eye. They can be spread through contact with infected blood. If they get into the bloodstream, an individual may become infected and sick.
- Most personnel cannot reasonably anticipate coming into contact with blood during their day-to- day work duties. That's why it's imperative that all personnel understand the danger of exposure to bloodborne pathogens and ways to minimize their risk.
- Bloodborne pathogens may be present in blood and other materials, such as:

Body fluids containing visible blood semen and vaginal secretions

Torn or loose skin

- Bloodborne pathogens can cause infection by entering the body through:
 - Open cuts and nicks skin abrasions dermatitis

Acne

Mucous membranes of the mouth, eyes or nose

SAFETY AND HEALTH PROGRAM

Workplace transmission

The most common bloodborne pathogens are HIV, Hepatitis B, and Hepatitis C:

HIV (AIDS)

HIV, the human immuno-deficiency virus, attacks the body's immune system causing it to weaken and become vulnerable to infections that can lead to a diagnosis of acquired immune deficiency syndrome or AIDS.

HIV is transmitted mainly through sexual contact and sharing contaminated needles, but also may be spread by contact with infected blood and body fluids. HIV is NOT transmitted indirectly by touching or working around people who are HIV-positive.

Employees can prevent getting HIV by stopping the passage of the virus from a person who has HIV to them. In many instances, the employee has control over the activities that can transmit HIV. Since HIV is most frequently transmitted by sharing needles or through sexual intercourse, employees can stop transmission by refusing to engage in these behaviors.

Hepatitis B

Hepatitis is a general term used to describe inflammation (swelling) of the liver. Alcohol, certain chemicals or drugs, and viruses such as hepatitis A, B, C, D, E and G may cause hepatitis.

Hepatitis B is a serious, sometimes fatal disease, caused by a virus that infects and attacks the liver. The virus is transmitted through direct contact with infected blood, semen, or vaginal fluid. It is primarily spread through sexual contact.

In studies that examine transmission following injections into the skin, HBV is 100 times more contagious than HIV.

HBV can also be transmitted indirectly because it can survive on surfaces dried and at room temperature for at least a week! That's why contaminated surfaces are a major factor in the spread of HBV.

Each year there are up to 200,000 new infections and 5,000 hepatitis B related deaths in the U.S. (compared to 40,000 new HIV infections per year).

One in approximately 20 persons now has, or will one day have, hepatitis B Transmission of hepatitis B is preventable:

- Use latex condoms during sex
- Do not share needles
- Use universal precautions in the workplace
- Get the hepatitis B vaccination

Hepatitis C

Hepatitis is a general term used to describe inflammation (swelling) of the liver. Alcohol, certain chemicals or drugs, and viruses such as hepatitis A, B, C, D, E and G may cause hepatitis.

Hepatitis C is a serious, often fatal disease, caused by a virus that infects and attacks the liver. HCV is more common than hepatitis B and ranks slightly below alcoholism as a cause of liver disease. However, HCV is not as infectious as HBV because there are generally lower levels of the hepatitis C virus in the blood than of the hepatitis B virus

HCV is primarily transmitted through blood-to-blood contact -- most commonly through shared needles. The risk of transmitting HCV through sexual contact appears to be low, but precautions should be taken anyway. HCV cannot be transmitted by casual contact such as shaking hands or sharing bathroom facilities.

SAFETY AND HEALTH PROGRAM

Up to 180,000 people may become infected with HCV each year in the U.S. Transmission of hepatitis C is preventable:

- Use latex condoms during sex
- Do not share needles
- Use universal precautions in the workplace

HOWEVER, unlike hepatitis B, currently there is NO VACCINE for hepatitis C. And also unlike HBV, there is no drug to prevent HCV infection after an exposure.

Guidelines for Handling Blood and Other Bodily Fluids:

Many personnel are concerned that HIV may be spread through contact with blood and other body fluids when an accident occurs at work.

HIV, as noted earlier, has been found in significant concentrations in blood, semen, vaginal secretions, and breast milk.

Other body fluids, such as feces, urine, vomit, nasal secretions, tears, sputum, sweat, and saliva do not transmit HIV unless they contain visible blood. However, these body fluids do contain potentially infectious germs from diseases other than AIDS.

If an individual has contact with any of these body fluids, they are at risk of infection from these germs. It should be remembered that the risk of transmission of these germs depends on many factors, including the type of fluid contacted, the type of contact made, and the duration of the contact.

Very simply, it is good hygiene policy to treat all spills of body fluids as infectious in order to protect personnel from becoming infected with any germs and viruses. The procedures outlined below offer protection from all types of infection, and should be followed routinely.

How Should Blood and Body Fluid Spills be Handled?

Whenever possible, employees shall wear disposable, waterproof gloves when they expect to come into direct hand contact with body fluids (when treating bloody noses, handling clothes soiled by incontinence, or cleaning small spills by hand). Gloves used for this purpose shall be put in a plastic bag or lined trash can, secured, and disposed of daily. Hands should always be washed after gloves are removed, even if the gloves appear to be intact.

If an employee has unexpected contact with body fluids or if gloves are not available (for example, applying pressure to a bleeding wound), the employee shall wash their hands and other affected skin for at least 30 seconds with soap and water after the direct contact has ended. This precaution is recommended to prevent exposure to other pathogens, not just HIV. As has been discussed, blood, semen, vaginal secretions, and blood-contaminated body fluids transmit HIV. Wiping a runny nose, saliva, or vomit does not pose a risk for HIV transmission.

Handwashing

Proper handwashing requires the use of soap and warm water and vigorous washing under a stream of running water for at least 30 seconds. If hands remain visibly soiled, more washing is required. Scrubbing hands with soap will suspend easily removable soil and microorganisms, allowing them to be washed off.

Running water is necessary to carry away dirt and debris. Rinse your hands under running water and dry them thoroughly with paper towels or a blow dryer. When hand-washing facilities are not available, use a waterless antiseptic cleanser, following the manufacturer's directions for use.

Disinfectants